Cancer treatment within 62 days

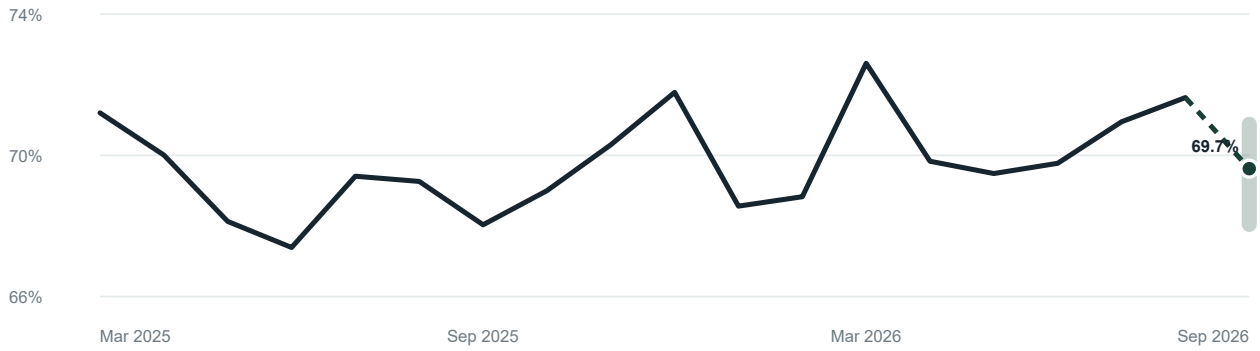
Forecast for September 2026 · data through August 2026

The latest Cancer treatment within 62 days result was 71.9%. For the next release, it is expected to worsen to 69.7%. The model's expected deterioration is mainly due to the usual month-to-month seasonal movement; the underlying trend is broadly flat. The current projection is off course for 80% by March 2027. Across providers, sustained favourable signals outnumber adverse ones (30 to 20).

LATEST · AUGUST 2026	FORECAST · SEPTEMBER 2026	MAR 2027 TARGET OUTLOOK
71.9%	**69.7%**	Projected **74.0%**
1.2pp above forecast	80% expected range 68.0%–71.1%	Target **≥80%**
		4% Model-implied chance

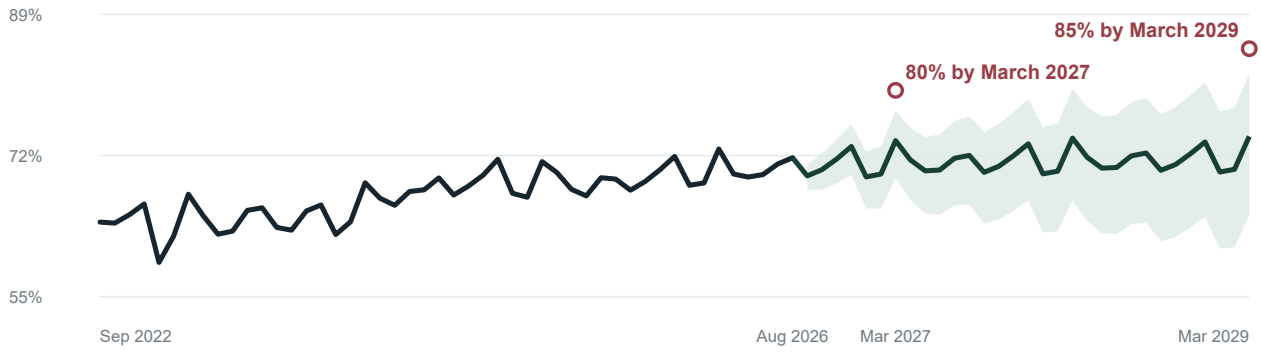
Recent performance and next release

The vertical band is the 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



Benchmark: 85% constitutional standard. 6.0pp below milestone

Data: NHS England cancer waiting times. **Targets:** Medium Term Planning Framework interim target · Medium Term Planning Framework final target

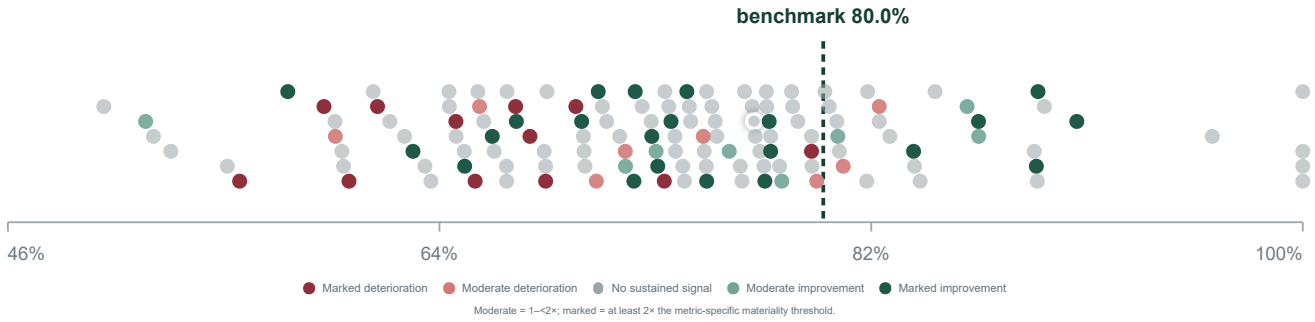
Method: Backtest-weighted ensemble of current-level seasonal movement, damped annual drift and a damped recent seasonal trend; at least 2 components are required. The final fit uses 53 consecutive months. Accuracy is tested on 24 rolling one-month-ahead forecasts. The 80% range uses empirical historical error quantiles from 24 forecast errors. Weights use inverse rolling RMSE and are shrunk toward equal weights. The pre-release commentary compares the current-level seasonal component with the combined forecast. Provider distribution and trajectory watch are shown on page 2.

Cancer treatment within 62 days

Distribution and trajectory watch · August 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



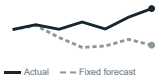
Moderate = 1–<2×; marked = at least 2× the metric-specific materiality threshold.

Trajectory watch

Six actual months are compared with one forecast path fixed before that window. Both lines begin at the same observed value. A low performer can be improving, while a high performer can be deteriorating. Status: Actual performance is compared with the monthly path forecast six months earlier. The displayed figure is the latest actual minus the fixed forecast for that same month; the six-month window checks persistence.

114	30	20	64
providers assessed	sustained favourable	sustained adverse	without an active signal

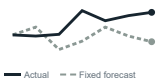
Largest sustained favourable moves

NORTH CUMBRIA INTEGRATED CARE NHS FOUNDATION TRUST
Latest 77.8% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


29.0pp
LATEST MONTH: ABOVE FORECAST

NORTH TEES AND HARTLEPOOL NHS FOUNDATION TRUST
Latest 62.9% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


15.2pp
LATEST MONTH: ABOVE FORECAST

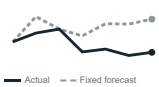
SALISBURY NHS FOUNDATION TRUST
Latest 73.1% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


15.1pp
LATEST MONTH: ABOVE FORECAST

Largest sustained adverse moves

GLOUCESTERSHIRE HOSPITALS NHS FOUNDATION TRUST
Latest 65.5% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


14.0pp
LATEST MONTH: BELOW FORECAST

BEDFORDSHIRE HOSPITALS NHS FOUNDATION TRUST
Latest 61.4% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


13.4pp
LATEST MONTH: BELOW FORECAST

SANDWELL AND WEST BIRMINGHAM HOSPITALS NHS TRUST
Latest 64.7% · 6 months compared with a path fixed six months earlier
Forecast fixed before the six-month window


12.9pp
LATEST MONTH: BELOW FORECAST

Trajectory-watch rule. Six actual months are compared with the corresponding path fixed before the six-month window. The displayed gap is the latest actual minus the fixed forecast for that same month. A signal requires a material latest-month gap and at least five of six months in that direction. These are screening prompts—not league tables, causal estimates or judgements of provider quality.